



From Learner to Leader: Comparing Approaches to Professionalism Lapses Across the Continuum



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DISCLOSURES



Learning Objectives

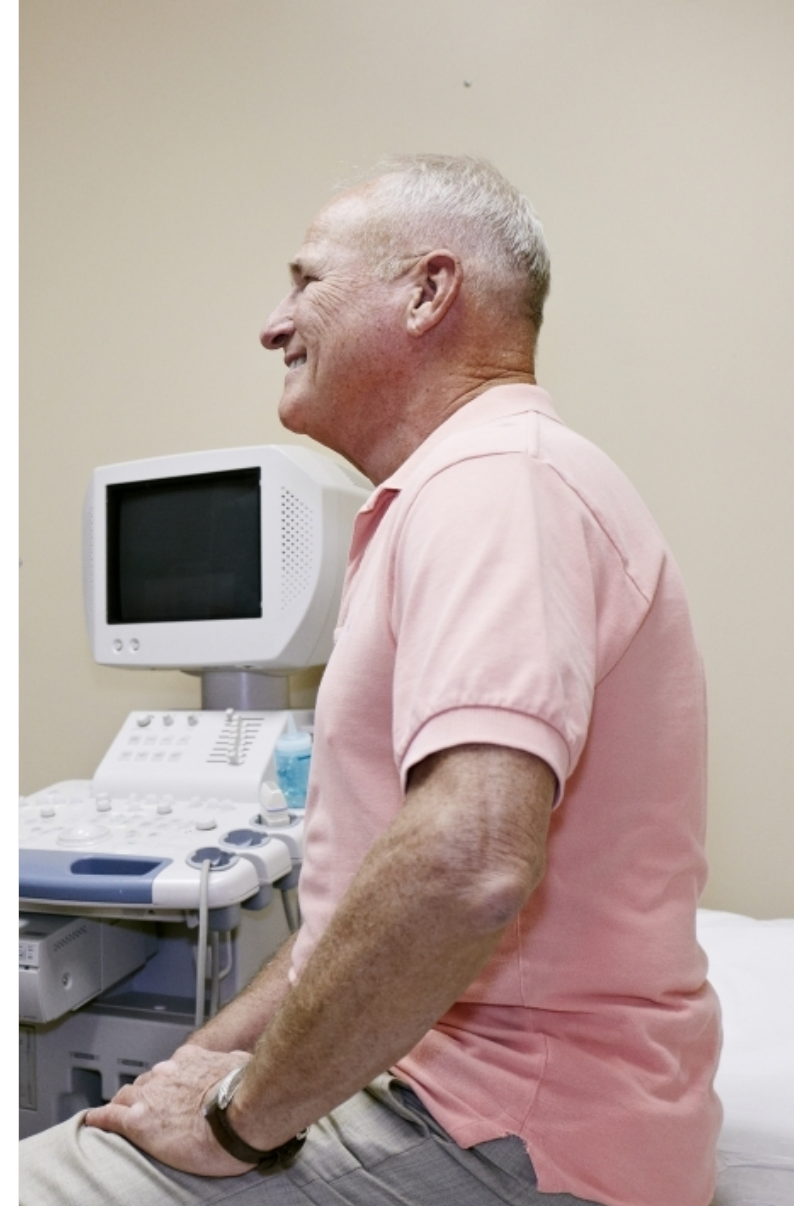
After this session, participants should be able to:

- Describe the link between early professionalism lapses and later certification or practice challenges.
- Analyze how contextual assessment and competency frameworks guide remediation decisions for students, residents and practicing physicians
- Apply cross-continuum insights to strengthen professionalism oversight and remediation policy

What is Professionalism ?

American Board of Medical Specialties:

“belief system in which group members (“professionals”) declare (“profess”) to each other and the public the shared competency standards and ethical values they promise to uphold in their work and what the public and individual patients can and should expect from medical professionals.”





Impact of positive professionalism

- Increased patient and community trust
- Improved patient safety and health outcomes
- Heightened sense of meaning and purpose
- Formative learning environments conducive to continuous improvement
- Improved satisfaction and morale of healthcare team members

Impact of unprofessional behavior



- Adverse effects on:
 - Team dynamics
 - Communication
 - Patient care
 - Public trust

“Fundamentally, disruptive behavior by individuals subverts the organization's ability to develop a culture of safety”

Professionalism Matters Across The Continuum Of Training And Practice

THE NEW ENGLAND JOURNAL of MEDICINE

SPECIAL ARTICLE

Disciplinary Action by Medical Boards and Prior Behavior in Medical School

Maxine A. Papadakis, M.D., Arianne Teherani, Ph.D., Mary A. Banach, Ph.D., M.P.H., Timothy R. Knettler, M.B.A., Susan L. Rattner, M.D., David T. Stern, M.D., Ph.D., J. Jon Veloski, M.S., and Carol S. Hodgson, Ph.D.

Multicenter Study > Acad Med. 2020 Jun;95(6):888-895. doi: 10.1097/ACM.0000000000003145.

Do Professionalism Lapses in Medical School Predict Problems in Residency and Clinical Practice?



Edward Krupat ¹, Jules L Dienstag, Susan L Padrino, John E Mayer Jr, Miles F Shore, Aaron Young, Humayun J Chaudhry, Stephen R Pelletier, Ben Y Reis

Affiliations + expand

JAMA
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Original Investigation | Medical Education

Trainee Physician Milestone Ratings and Patient Complaints in Early Posttraining Practice

Misop Han, MD, MS; Stanley J. Hamstra, PhD; Sean O. Hogan, PhD; Eric Holmboe, MD; Kelly Harris, MD; Eric Wallen, MD; Gerald Hickson, MD; Kyla P. Terhune, MD, MBA; Donald W. Brady, MD; Bruce Trock, PhD; Kenji Yamazaki, PhD; Jessica L. Bienstock, MD, MPH; Henry J. Domenico, MS; William O. Cooper, MD, MPH



Addressing Professionalism Across Career Stages

The internal medicine clerkship director asks you to meet with a medical student; he is frequently **late** to morning rounds, and “**disappears**” for long periods of time in the afternoon.

Before starting clinical rotations, the student was reported as unprofessional due to **lack of responsiveness to emails** from course coordinators, and **late submission of assignments**.

The student has received consistently **high praise** for his medical **knowledge** and **clinical performance**, and he is well liked by his peers.



UME Considerations



Identify concerning behaviors

- Gather data
- Understanding the reporter's observations
- Begin to determine contributing factors
- Determine previous feedback and opportunity to address

Explore contributing factors

- Variety of possible root causes
 - Can include:
 - Skills and executive functioning gaps
 - Role of self-awareness
 - Learning challenges
 - Health and well-being
 - System-based factors
 - Personal and social stressors

Suggested approach to this case

Suggested Approach

- Understanding
 - Initiating dialogue and building rapport
 - Initial assessment
 - Understanding the impact and motivation to change
- Action
 - Identifying actionable goals
 - Based on root cause, approach referrals
 - Follow-up and ongoing support





residency in obstetrics and Gynecology.

The program director receives complaints about the resident from supervising faculty. He is **late in responding to Epic in-basket messages**, leading to delays in patient care.

The resident **requires frequent reminders** from the program leadership **to log work hours and surgical cases**.

Before meeting with the resident, you review his clinical evaluations; he is described as “**going the extra mile for his patients**.”

GME Considerations



Identify concerning behaviors

- Are expectations clear?
- Triangulating sources of information
- Exploring if the trainee has received direct feedback

Explore contributing factors

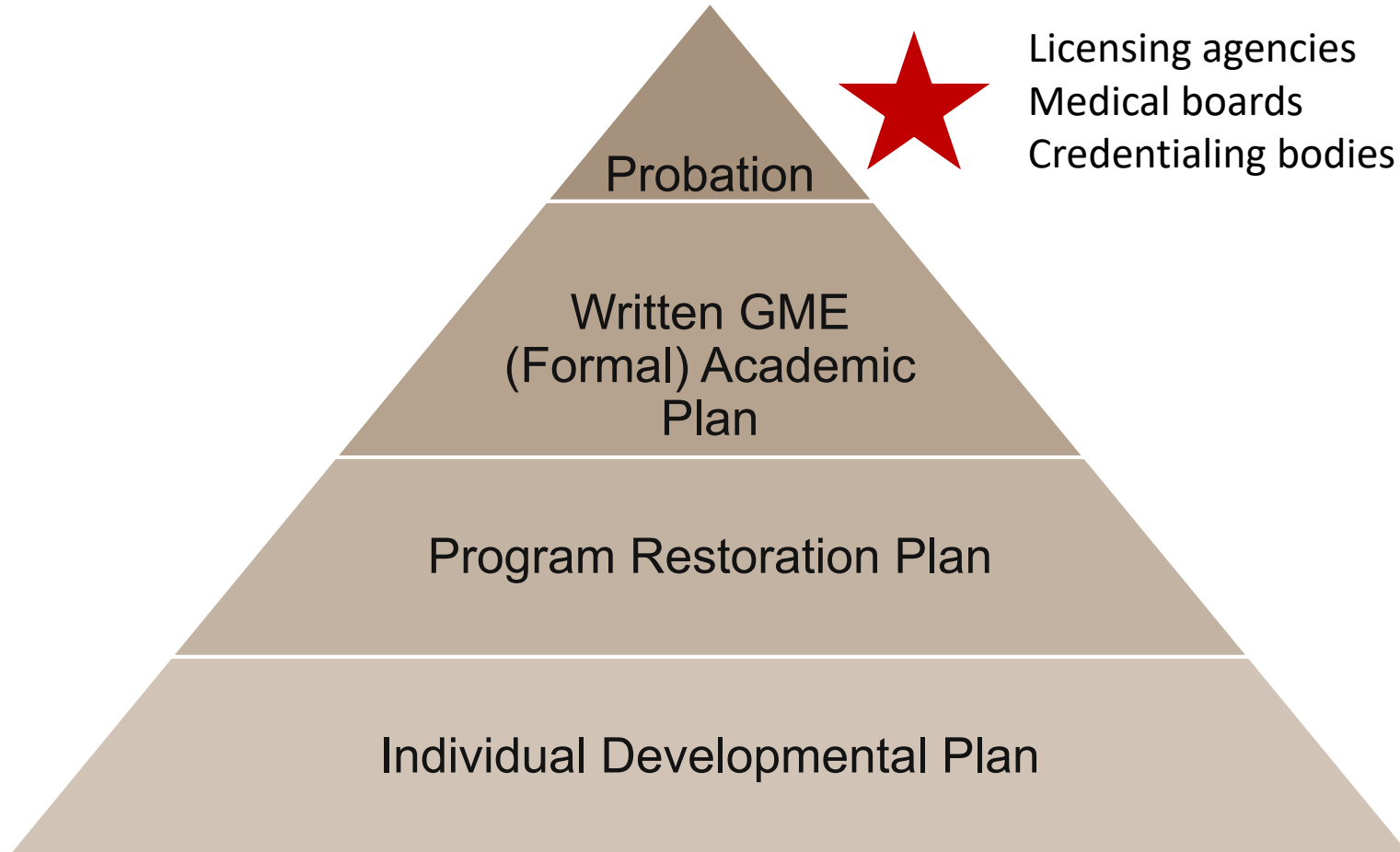
- *Individual:*
 - Time management
 - Burnout
 - External stressors
- *System:*
 - Workload
 - Resources

ACGME PROFESSIONALISM MILESTONES

Professionalism 3: Accountability/Conscientiousness

Level 1	Level 2	Level 3	Level 4	Level 5
Takes responsibility for failure to complete tasks and responsibilities, identifies potential contributing factors, and describes strategies for ensuring timely task completion in the future	Performs tasks and responsibilities in a timely manner with appropriate attention to detail in routine situations	Performs tasks and responsibilities in a timely manner with appropriate attention to detail in complex or stressful situations	Recognizes situations that may impact others' ability to complete tasks and responsibilities in a timely manner	Recognizes one's own role in leading the care of all patients on the service while mentoring/coaching other team members to ensure the best possible care of patients, including prioritizing tasks and mitigating burnout

Escalation from Program to Institution

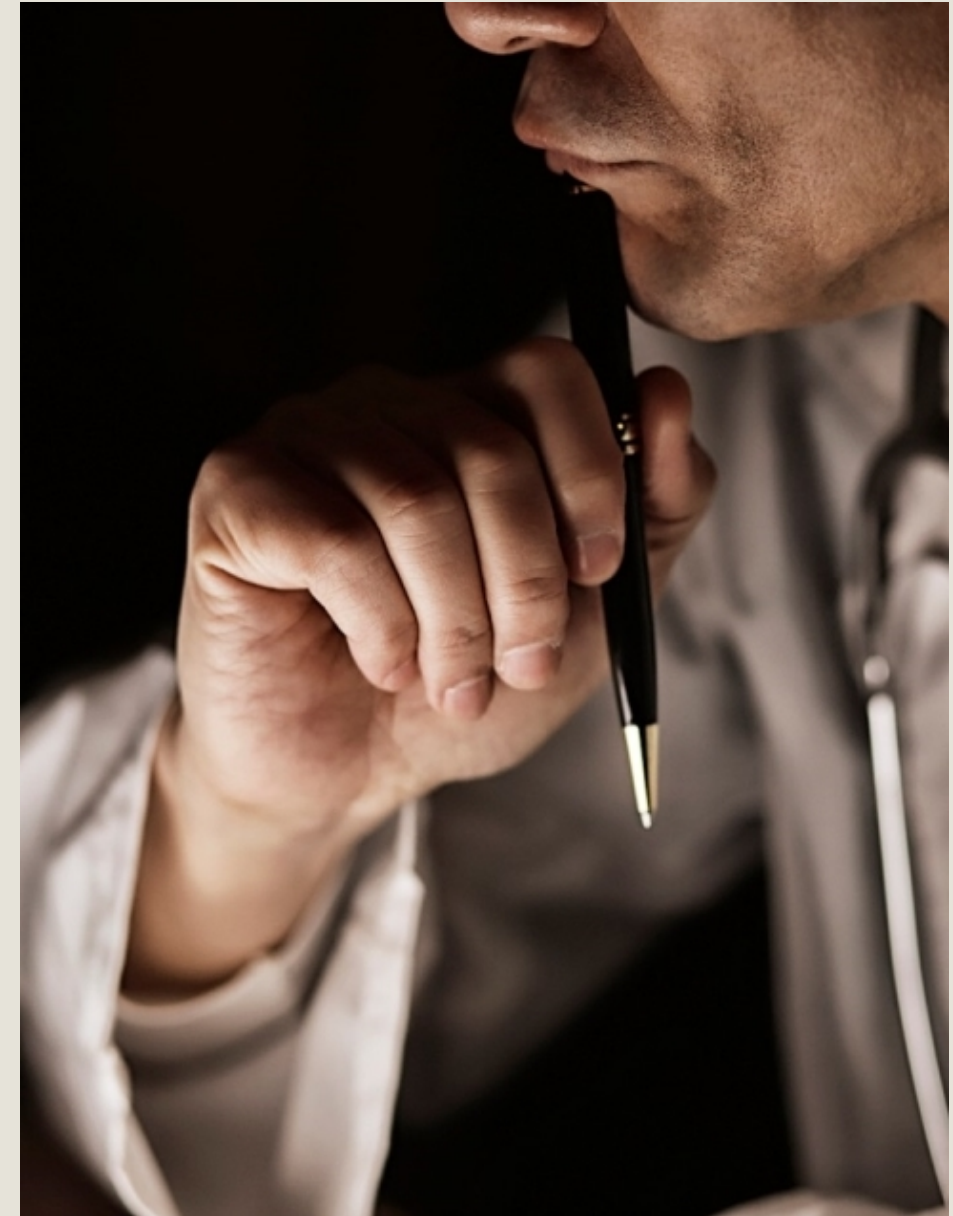


The resident graduates and is hired as faculty at an academic health center.

Six months into practice, he is called into the Chair's office due to several complaints from trainees and colleagues.

He is **behind on patient charts** and has been **delayed in submitting evaluations** of both students and residents.

He is noted to be an excellent surgeon, but there are also reports of “**raising his voice**” in the operating room and **belittling** junior residents.



Faculty Considerations



Identify concerning behaviors

- Are expectations clear?

Explore contributing factors

- *Individual* - Overwhelmed? Lack of confidence? Personal stressors?
- *Systems* - Lack of resources?

Suggested approach to this case



Describe problematic **behaviors**

Promote self-reflection using a **competency-based framework**

Acknowledge **systems** contributions, while identifying opportunities for **individual** growth

Develop specific **goals**, necessary **resources**, and plan for **follow up**

AAMC Premed professionalism competencies

PROFESSIONAL COMPETENCIES

-  **Commitment to Learning and Growth**
Practices continuous personal and professional growth for improvement, including setting and communicating goals for learning and development; reflects on successes, challenges, and mistakes; pursues opportunities to improve knowledge and understanding; and asks for and incorporates feedback to learn and grow.
-  **Empathy and Compassion**
Recognizes, understands, and acknowledges others' experiences, feelings, perspectives, and reactions to situations; is sensitive to others' needs and feelings; and demonstrates a desire to help others and alleviate others' distress.
-  **Ethical Responsibility to Self and Others**
Behaves with honesty and integrity; considers multiple and/or conflicting principles and values to inform decisions; adheres to ethical principles when carrying out professional obligations; resists pressure to engage in unethical behavior; and encourages others to behave honestly and ethically.
-  **Interpersonal Skills**
Demonstrates an awareness of how social and behavioral cues affect people's interactions and behaviors; adjusts behaviors appropriately in response to these cues; recognizes and manages one's emotions and understands how emotions impact others or a situation; and treats others with dignity, courtesy, and respect.
-  **Oral Communication**
Effectively conveys information to others using spoken words and sentences; actively listens to understand the meaning and intent behind what others say; and recognizes potential communication barriers and adjusts approach or clarifies information as needed.



Example
competency
framework to
facilitate self-
reflection on
strengths and
areas for
improvement

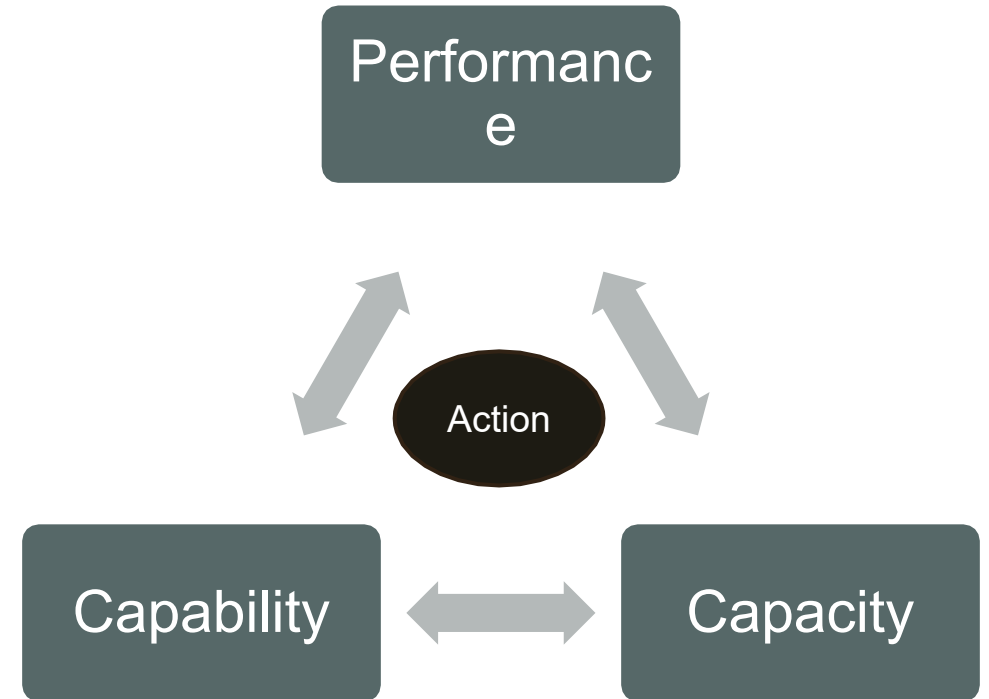
Shared Principles Across the Continuum (UME, GME, faculty)

- **Fairness, transparency, and proportionality** are foundational values guiding professionalism remediation at all career stages
- **Gather collateral data** to determine acute versus chronic pattern of behavior, whether there were prior attempts to address and what was the outcome
- At all stages, **early identification and intervention is critical** to optimize safety (patients, healthcare team members, identified individual)
- Be **curious** about the behavior and potential contributory factors



EVLA Framework: Understanding Contributory Factors to professional lapses

- **Capacity**
 - Biopsychosocial Factors
- **Capability**
 - Cognitive/emotional requirements of task
- **Performance**
 - Readiness
 - Ability to recognize / manage triggers
- **Action**
 - Ability to respond in the moment

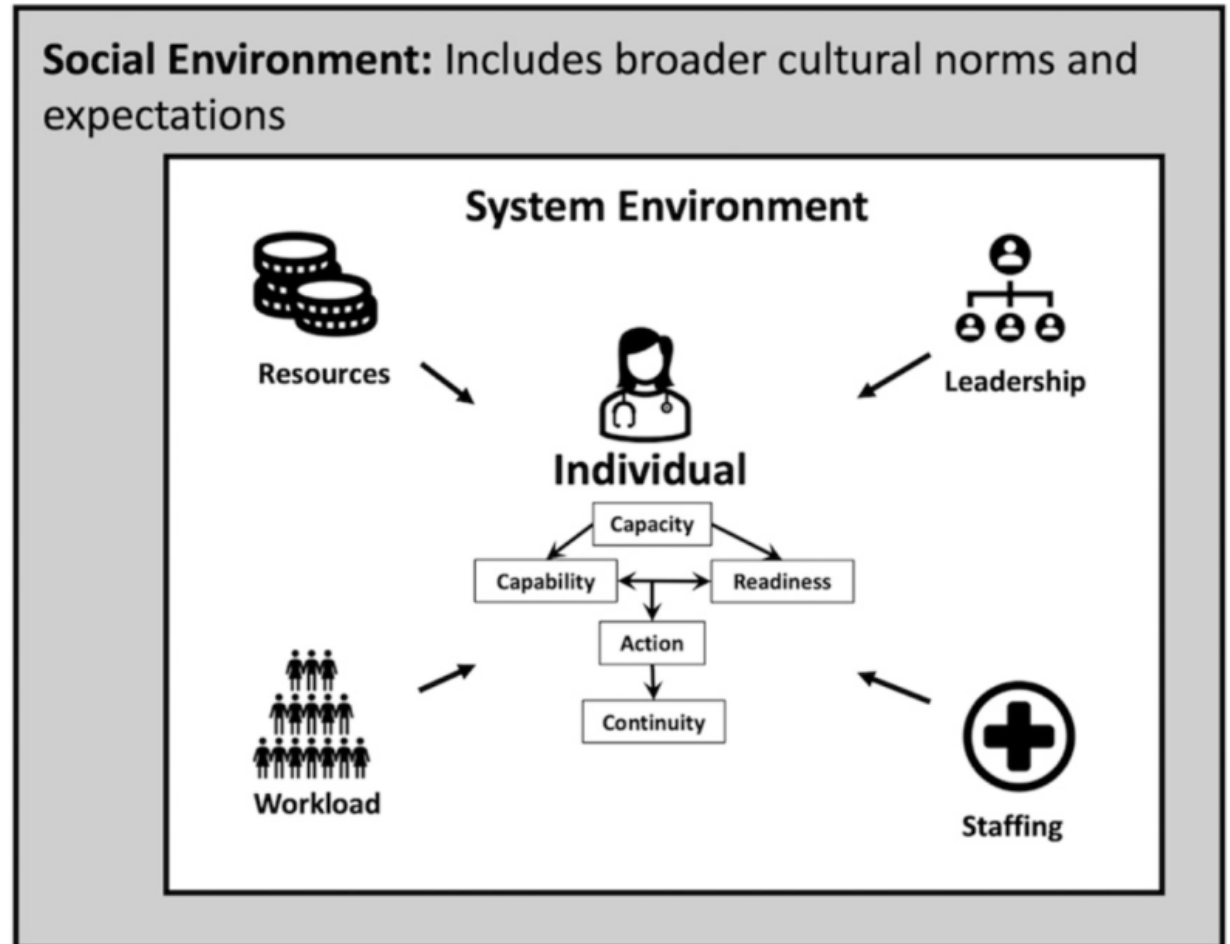


Williams, B. W and Williams, M. (2020) Understanding and Remediating Lapses in Professionalism: Lessons from the Island of Last Resort. *Annals of Thoracic Surgery*; 109:317-324.

*EVLA: Environmentally Valid Learning Approach

Personal and Systemic Factors Contributing to Professionalism Lapses

Acknowledgement and recognition of the **factors that contribute to unprofessional behavior** are also crucial in investigating professionalism concerns



Institutional Variability and the Iceberg Framework



Whether student, resident, faculty **there may be more than meets the eye**

Be curious – look underneath the tip of the iceberg-what are the causes, what has been tried, what has worked, what has not worked

Identify needed treatment (medical and or mental health) and **remediation** (skills development)

Ongoing **assessment** of compliance and progress are critical

Ongoing **feedback** is critical

Consider **institution-specific** factors and resources

Case-Based Discussions

Discuss each case from your
table's vantage point
(UME, GME or faculty) (5 min)

Designate an individual from
your table for report-out (5 min)

Case 1

During rounds, the **third-year medical student** presents a patient outside the room and shares that he had used AI to develop the differential. He then told the patient and family about the differential and recommended they pursue diagnostic testing.

The **resident** rolls her eyes and says, “Great, another AI doctor.”

The **faculty member** weighs whether or not to address as he has 15 other patients to round on.



Case 1



Is the behavior of the student / resident / faculty unprofessional?

What additional information would be relevant?

Case 1 Report-out

Case 2

- After an overnight call, “X” presents a patient to the oncoming day team. The sign-out is that the patient was stable overnight, without any issues.
- On daytime rounds, you are notified by the nurse that the patient had several episodes of hypotension overnight as documented in the chart.



Case 2

Is this behavior unprofessional?

What additional information would be relevant?



Case 2 Report-out



Putting it all together

Focus on the behavior(s) and their consequences, not the person

Consider the **context** and stage of training; ensure expectations are clear

Link expectations to established **competency** frameworks

Be curious about **contributing factors** – individual, systems-level

Early intervention and ongoing feedback are critical

Thank you for your attention and
engagement



Questions?

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